

GROUP NAME: MOREMI

COHORT: 10

CHALLENGE: 4

NAME OF ACTIVE MEMBERS:

- Adegboyega Babatunde
- Ageebee Thomas
- Elufisan Iyiola
- Faruq Olasupo
- Salim Alhassan
- Osumune Chiamaka

Stakeholder Engagement Plan

1. Stakeholder Organisations

Group A: Frontline Healthcare Professionals (Primary Care Physicians, CHEWs, and Nurses)

They are community health extension workers and clinical professionals who provide point-of-care services at primary healthcare institutions.

Why they matter: The project's practical utility depends on end users' everyday adoption. Their subject knowledge is crucial for guaranteeing safe use and confirming the system's clinical workflow integration.

Their top priorities include speed, diagnostic precision, ease of use offline, minimal administrative work, and protection from therapeutic error or professional responsibility.

Group B: Community Leaders and Patient Advocacy Organisations

They are grassroots community leaders, members of local health committees, and representatives of patient advocacy groups who speak for the many groups receiving care.

Why they matter: They serve as guardians and speak for the eventual beneficiaries.

2. Potential Benefits and Risks

Stakeholder Group	Potential Benefits	Potential Harms & Concerns	Ethical, Social, Cultural & Economic Considerations
Frontline Healthcare Workers	Reduced literature search time, reduced burnout, real-time clinical decision support, and standardised diagnostic guidance.	Risk of automation bias, alert fatigue, workflow disruption, or operational delay if the system generates overly complex outputs.	Professional Responsibility: Protecting clinician agency and ensuring the AI remains strictly advisory so liability remains clear.
Patients & Community	Clearer health explanations, reduced diagnostic delays, improved health literacy, and equitable care delivery.	Misinterpretation of medical terms, privacy leaks through data capture, or anxiety caused by automated clinical messaging.	Equity & Privacy: Preventing demographic bias in recommendations, respecting local cultural health beliefs, and maintaining strict de-identification.

3. Engagements Methods

For Frontline Healthcare Workers: Participatory Design

- Why it's appropriate: Rather than treating clinicians as passive users, Participatory Design actively incorporates them in co-creating the interface and workflow mechanics.
- Suitability: Direct co-design guarantees that the tool fits seamlessly into hectic sessions because healthcare professionals have limited timetables and particular clinical practices.

- Execution: Use interactive wireframes and clinical scenario walkthroughs to conduct brief, practical co-design sprints. Clinicians evaluate literature synthesis formats, test simulated symptom inputs, and create the interaction flow to validate or override model recommendations.

For Patient Advocacy & Community Leaders: Deliberative Workshops

- Why it's appropriate: Deliberative workshops give community members an organised setting to discuss privacy, fairness, and communication standards while reviewing information on the AI tool and weighing trade-offs.
- Suitability: To address systemic concerns about trust and ethics and to comprehend the possibilities of the system, community leaders need an open, instructional environment.
- Execution: Host a series of half-day facilitated sessions. Present non-technical overviews of how the SLM operates, demonstrate sample patient-facing summaries, and facilitate group discussions on data privacy, acceptable communication tone, and local cultural nuances in health advice.

4. Influence on Project Design

- Dataset: Target additions to the retrieval corpus and dataset refinement to lessen demographic bias will result from deliberative workshops with community leaders that identify underrepresented local health conditions, regional terminology, and cultural phrasing.
- Model Design & Guardrails: The Retrieval-Augmented Generation (RAG) thresholds, which specify how rigidly the model must refrain from responding when confidence or evidence coverage is low, will be directly shaped by participatory design input from healthcare professionals.
- Results: The text-simplification logic for patient-facing explanations will be guided by insights from community workshops, guaranteeing that the language is accessible, culturally sensitive, and set at a suitable reading level.
- Evaluation: Realistic clinical scenario benchmarks and red-teaming criteria will be defined based on feedback from clinician co-design sessions. Point-of-care utility and safety will replace traditional accuracy scores as evaluation metrics.
- Deployment Decisions: User experience feedback during Participatory Design will inform offline deployment choices, such as optimising model quantisation so the SLM runs efficiently on low-cost local hardware without relying on cloud connectivity.